Category 2 ambulance response

Outturn for October 2026 · forecast issued with data through September 2026

The published result was 32 min 45 sec. It worsened 1.4 min from the previous result and was 1.0 min better than outlook · inside the 80% range.

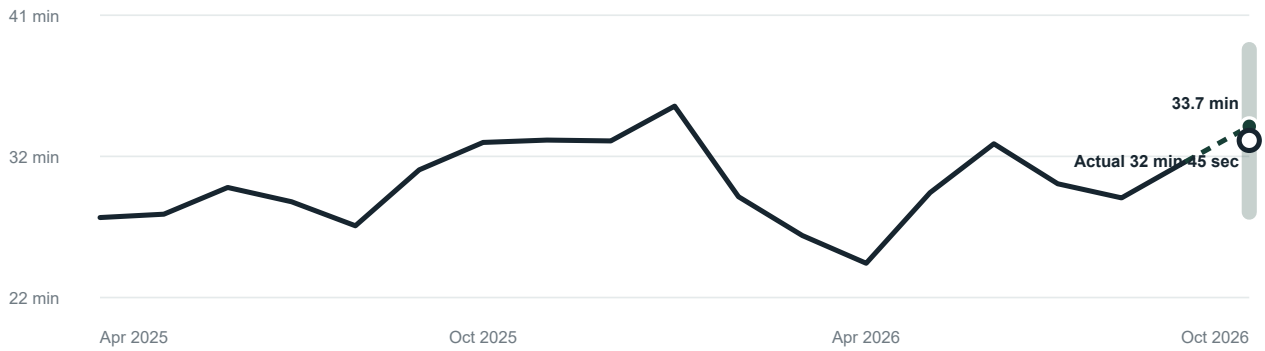
AT FORECAST · SEPTEMBER 2026	FORECAST · OCTOBER 2026	ACTUAL
31.3 min	**33.7 min**	**32 min 45 sec**
Known when the forecast was made	80% expected range 27.9 min–38.9 min	Worsened 1.4 min from the previous result · 1.0 min better than outlook · inside the 80% range

Off course · 2026/27 financial-year average: 25 minutes

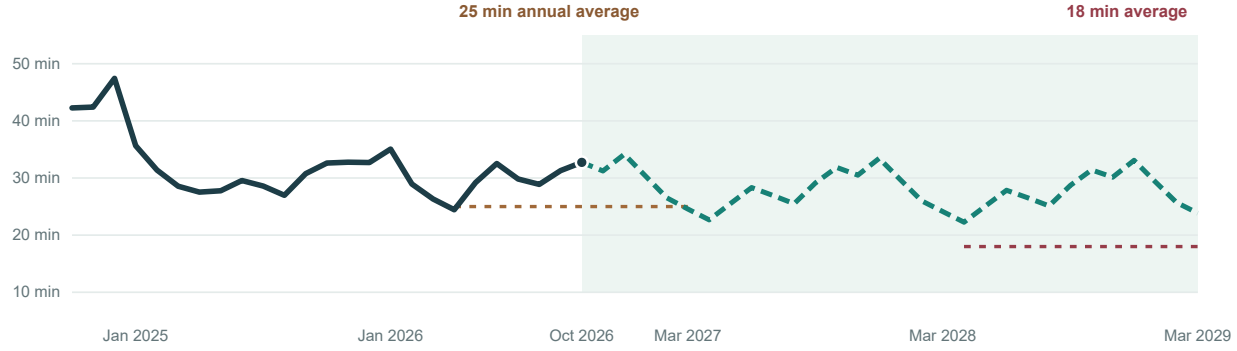
2026/27 annual-average outlook Projected **29.7 min** Target **≤25 min** Model-implied chance **2%**

Recent performance and current release

The vertical band is the 80% expected range.



Longer-term outlook and milestones · Monthly response times



Monthly response times. Targets: 25 min annual average in 2026/27; 18 min average by March 2029.

Targets: 25-minute annual average in 2026/27; 18-minute average response time by March 2029; 90% within 40 minutes by March 2029.

Data: NHS England Ambulance Quality Indicators. **Targets:** Medium Term Planning Framework interim target · Medium Term Planning Framework final target

Method: Backtest-weighted ensemble of current-level seasonal movement, damped annual drift and a damped recent seasonal trend; at least 2 components are required. The final fit uses 111 consecutive months. Accuracy is tested on 36 rolling one-month-ahead forecasts. The 80% range uses empirical historical error quantiles from 36 forecast errors. Weights use inverse rolling RMSE and are shrunk toward equal weights. The pre-release commentary compares the current-level seasonal component with the combined forecast. Provider distribution and trajectory watch are shown on page 2. Rolling averages use national AmbSYS A10/A30 totals (or eleven complete trust totals where needed) and require 12 complete months. Future incident counts are projected; annual target probabilities use correlated forecast errors.

Category 2 ambulance response

Distribution and trajectory watch · October 2026

Provider distribution

Latest reported performance across providers. This is a comparison of current levels, not a measure of whether each provider is improving or deteriorating.



Moderate = 1–<2×; marked = at least 2× the metric-specific materiality threshold.

Trajectory watch

Six actual months are compared with one forecast path fixed before that window. Both lines begin at the same observed value. A low performer can be improving, while a high performer can be deteriorating. Status: Actual performance is compared with the monthly path forecast six months earlier. The displayed figure is the latest actual minus the fixed forecast for that same month; the six-month window checks persistence.

11	1	3	7
providers assessed	sustained favourable	sustained adverse	without an active signal

Largest sustained favourable moves

ISLE OF WIGHT NHS TRUST

Latest 25.0 min · 6 months compared with a path fixed six months earlier

Forecast fixed before the six-month window



10.6 min

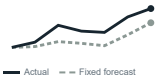
LATEST MONTH: BELOW FORECAST

Largest sustained adverse moves

EAST OF ENGLAND AMBULANCE SERVICE NHS TRUST

Latest 45.2 min · 6 months compared with a path fixed six months earlier

Forecast fixed before the six-month window



6.5 min

LATEST MONTH: ABOVE FORECAST

SOUTH WESTERN AMBULANCE SERVICE NHS FOUNDATION TRUST

Latest 42.3 min · 6 months compared with a path fixed six months earlier

Forecast fixed before the six-month window



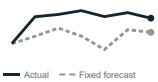
4.3 min

LATEST MONTH: ABOVE FORECAST

LONDON AMBULANCE SERVICE NHS TRUST

Latest 31.9 min · 6 months compared with a path fixed six months earlier

Forecast fixed before the six-month window



4.0 min

LATEST MONTH: ABOVE FORECAST

Trajectory-watch rule. Six actual months are compared with the corresponding path fixed before the six-month window. The displayed gap is the latest actual minus the fixed forecast for that same month. A signal requires a material latest-month gap and at least five of six months in that direction. These are screening prompts—not league tables, causal estimates or judgements of provider quality.